

جوستاف روسي انترناسيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : فرحه اميرهم عبيد اسحق

المحافظة : القاهرة

الحى / المدينة : المطريه

العنوان : 10 ش الحاج لبيب - عزبه النخل

تاريخ تسجيل الإستمارة : 19-09-2026

رقم البطاقة : 26201012502706 رقم التليفون : 01281299551

العمر (تاريخ الميلاد) : 01-01-1962 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
التوقيع: (_____)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : جراحات أورام تدخل جراحى لاستئصال تحظى وتكميلي ورفع للثدى **قسم العلاج :** +استئصال الغدد الليمفاوية و/او الغدد الحارسة [13156]
الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

مدير عام

مديرية الشئون الصحية

رئیس

المجلس الطبي المتخصص

ختم

شعار الجمهورية

--

Name: فرحة اميرهم عبيد اسحق National ID: 26201012502706	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

Breast-conserving and completion surgery with mastopexy plus axillary lymph node dissection and/or sentinel lymph node biopsy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستاف
روني
مدير عام العمليات

مراجعة
9-14



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
طبية
مختصة

09/13/2026

MDT FORM

Name: فرحة اميرهم عبيد اسحق	MRN: 13229
Age: 65 Y	ID: 26201012502706
Menopausal Sta: post	Phone: * 01281299551
NRS SCORE:	Marital Status: married
BMI:	Family History: -ve
Gender:	Reason Of MDT:
CO Morbidity: DM , HTN , cardiac	New
Surgical History: * NAD	ECOG Performance Status 0 0 1 2 3 4

Clinical Data:

BX: IDC 2 with mucinous production
ER: 8/8 **PR:** 8/8 **HER2:** 0 **KI67:** 20%

History:

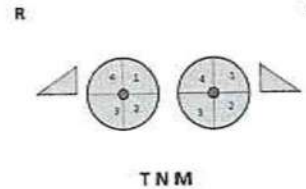
Sonomammography:
 Rt Para areolar lesion invading skin 3 x 2.7 x 2 cm + enlarged LNs cortical thickness 3mm

Biomarker/Molecular

Mets Workup:
 CT cap D 10 hemangioma
 L 4-5 degree degeneration

Recommendations:
 by examination skin overlying mass not pinchable but not fungating

MDT Outcome & Plan
 UPFRONT RT CENTRAL QUADRENTECTOMY VS SM AFTER COUNCELING PT AND SLNB



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:
 T 3 N 1 M0

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د محمد كمالى	د محمد تيرايوي		

جمهورية مصر العربية
مطابقة لمقتضى الشريعة الإسلامية

فرحة
أميرهم عبيد اسحق
١٠ ش. الحاج لبيب - عزبة النخل
المطرية - القاهرة

٢٦٢.١.١٢٥.٢٧.٦

JD4281021

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عيسى

٢٠٢١/١١ ٢٦٢.١.١٢٥.٢٧.٦
ربة منزل

انثى
مسيحية
متزوجة
عاطى الفنى اسكندر بولس
البطاقة سارية حتى ٢٠٢٨/١١/٢٨



Women's Health Outreach Program
Bilateral Full field Digital Mammography

Name: Farha Ibrahim Abeid

Date: 21-07-2026

Clinical summary: 64 years old lady with right breast lump.

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Scattered fibroglandular elements (ACR density B).
- Right breast shows inner central lobulated dense parareolar lesion.
- **Ultrasound**
- Right breast para-areolar globular lobulated heterogenous iso to hypoechoic lesion with mild internal vascularity by color doppler is seen invading the overlying skin is seen at 3 o'clock position measures about 3 x 2.7 x 2 cm
- Prominent retroareolar ducts.
- Right enlarged axillary lymph nodes with diffuse cortical thickness reaching up to 3 mm in thickness with preserved central fatty hila.

Left breast

- Scattered fibroglandular elements (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound:**
- No solid or cystic lesions.
- Mildly dilated retroareolar ducts reaching up to 3.2 mm.
- Normal skin, nipple/areola complex.
- Non-specific axillary LNs.

Impression:

Right breast (BIRADS 4c): highly suspicious para-areolar lesion as described for ultrasound guided biopsy.

Left breast (BIRADS 2): mild duct ectasia.

Best regards,
Dr. Ayat Nadi, FEBR, ABR

PATHOLOGY DEPARTMENT
HISTOPATHOLOGICAL REPORT

Patient name: فرحة اميرهم عبيد اسحق

Age: 64 years

Sex: female

Request type: مبادره

Sub request type:

Hospital no:

Pathology no.: 728-2026

Department: صحه مرأه

Date of result: 8/8/2026

Referred by: د/امل

Date of biopsy: 29/7/2026

Nature of specimen: Tru-cut biopsy

I.D. Number: 26201012502706

Clinical diagnosis: Right breast mass

Gross pathology :

Multiple fragmented grayish white tissue cores up to 0.5x0.4cm long, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against ER- PR- Her2- Ki67

Microscopic picture: Section examined revealed core of tumor showing invasive duct carcinoma. The tumor forms nests, tubule and cribriform pattern infiltrating among focally desmoplastic stroma with mucous production (40% of tumor surface area). Tumor cells showing moderate anaplasia and moderate tendency to form tubules. There is moderate number of mitotic figures.

Immunohistochemical Result

ER: Strong nuclear staining in 80% of tumor cells (Allred Score 8/8).

PR: Strong nuclear staining in 80% of tumor cells (Allred Score 8/8).

Her2: Faint incomplete membranous staining in <10% of tumor cells (score 0) extras low.

Ki67: Positive nuclear staining of 20% of tumor cells.

Diagnosis: Right breast mass, tru-cut biopsy:

Invasive duct carcinoma, Grade II with mucous production (40%).

Signature:

Dr.Mona Mohamed.M.D



Pathology Lab. Manager

BASMA REFAAT M.D

Patient's Name: Farha Amirham Obeid Ishaq
Date: 2026-08-24
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Examination:

■ **Computed tomography of the chest, abdomen and pelvis**

Technical data:

- 5 mm CT scans of the abdomen and pelvis with IV contrast

Clinical data: newly diagnosed right breast cancer

Comparison: none

Findings:

▪ **Chest findings:**

- Right breast retroareolar soft tissue density lobulated lesion 2.4 cm with overlying skin thickening.
- Clear both lungs fields with no active parenchymatous infiltrates.
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Average cardiac size, no pericardial effusion.

▪ **Abdomen and pelvis findings:**

- The liver shows enlarged size with smooth contours, diffuse low hepatic parenchyma density (enlarged fatty liver) and no evidence of focal lesions.
- No intra hepatic biliary dilatation.
- Normal caliber and homogenous enhancement of the portal vein.
- The spleen shows normal size. No evidence of focal lesions.
- Normal CT appearance of the gall bladder, pancreas and adrenal glands
- Both kidneys are of normal site, size and shape, no stones, back pressure or masses bilaterally, right renal small simple cortical cyst.
- The urinary bladder shows average distention, no wall thickening, no stones, diverticulae or masses.
- Bulky uterus with contour bulges/ calcified lesions possible fibroids
- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction
- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection
- D10 vertebral body lesion possible hemangioma
- L4-5 1st degree degenerative anterolithesis sequel to facet joints arthropathy

Opinion:

- **Right breast lesion as described for dedicated breast studies correlation**
- **No definite pulmonary nodules or masses**
- **No suspicious abdominal or pelvis metastatic lesions on CT base**
- **Enlarged fatty liver**
- **D10 vertebral body lesion possible hemangioma**
- **L4-5 1st degree degenerative anterolithesis sequel to facet joints**

9/13/26, 1:09 PM

code:09 -- Name_A MDT FORM -- Name_E MDT FORM

جوسناف
رئيسية
مستشفى
مصر٢٠٢٤
٩-١١مبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةالهيئة العامة
للصحة
مصر

09/13/2026

□

MDT FORM

Name:	فرحة احمد عبد الحق	MRN:	13229
Age:	65 Y	ID:	26201012502706
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BMI:		Family History:	-ve
Gender		Reason Of MDT:	
CO Morbidity:	DM , HTN , cardiac	New	
Surgical History:*	MAD	ECOG Performance Status	☐ 0 ☐ 1 ☒ 2 ☐ 3 ☐ 4

Clinical Data:

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د محمد كمال	د محمد نور دوي		

٢٠٢٤
٩-١١



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28
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core.
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Signature:
Dr.Mona Mohamed.M.D

Me



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Farha Amirham Obeid Ishaq

Date:

2026-08-24

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

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